LABORATORY REPORT

Name	: **Mr.SATENDRA KUMAR**	Location	: KPL A43
Age/Gender	: **31 Y(s) /Male**	Registered On	: 11-08-2025 14:42
Reg No	: 1108253311	Collected On	: 11-08-2025 14:42
Lab ID No	: KP1002545	Reported On	: 11-08-2025 16:36
Sample ID	: 601613195	Referred By	: LBSH
Sample Type	: EDTA Whole Blood	Client Name	: MOOVE HEALTH CARE
		Processed At	: KPL A43

Test	Result	Unit	Reference Range
COMPLETE BLOOD COUNT (CBC) WHOLE BLOOD (EDTA)			
HAEMOGLOBIN	: **10.00**	g/dl	13.0 - 17.0
Method : Photometric			
HAEMATOCRIT	: **32.60**	%	40.0 - 50.0
Method : Cumulative Pulse Height Detection			
R.B.C COUNT	: **4.31**	x10^12/L	4.50 - 5.50
Method : Electrical Impedence			
RDW-CV	: **15.20**	%	11.6 - 14.0
Method : Calculated Parameter			
M C V	: **75.60**	fL	83.0 - 101.0
Method : Calculated Parameter			
M C H	: **23.20**	pg	27.0 - 32.0
Method : Calculated Parameter			
M C H C	: **30.70**	g/dl	31.5 - 34.5
Method : Calculated Parameter			
PLATELET COUNT	: 448.00	x10^9/L	150 - 450
Method : Electrical Impedence			
TLC	: **14.24**	x10^9/L	4.00 - 10.00
Method : Electrical Impedence			
DIFFERENTIAL LEUCOCYTE COUNT (DLC)			
Neutrophils	: 74.0	%	40.0 - 80.0
Lymphocytes	: **14.2**	%	20.0 - 40.0
Monocytes	: 10.0	%	2.0 - 10.0
Eosinophils	: 1.1	%	1.0 - 6.0
Basophils	: 0.7	%	0.0 - 2.0
ABSOLUTE NEUTROPHIL COUNT	: **10.54**	x10^9/L	2.00 - 7.00

MC - 2712

Dr.N.Magoon
M.D. (Path)
Consulting Pathologist DMC
Reg.No-97859

Print Date : 12-08-2025 16:18

"To Avail PHYSIOTHERAPY/ ECG/ NURSING SERVICES at your HOME, Call 7239-940000"

ON PANEL : **CGHS**, CAPF, DGEHS, ECHS, BHEL, INDIAN RAILWAY, NDMC, DELHI JAL BOARD, ONGC, NTPC, SAIL, BSF, SSB, CRPF, CISF, NSG, Assam Rifles

1. All reports are for interpretation by the treating doctor only and have to be viewed and correlated with clinical examination and other investivations. 2. All investivations have their limitation which is imposed by the limits to sensitivity and specificity of individual assay procedures as well as the quality of the specimen received by the laboratory. Isolated laboratory investigations never confirm the final diagnosis of the disease. 3. If the result (s) of tests are alarming or unexpected the doctor is advised to contact the lab immediately for needful and necessary action.

This Report is not subject to use for any medico-legal purpose

LABORATORY REPORT

Name	: **Mr.SATENDRA KUMAR**	Location	: KPL A43
Age/Gender	: **31 Y(s) /Male**	Registered On	: 11-08-2025 14:42
Reg No	: 1108253311	Collected On	: **11-08-2025 14:42**
Lab ID No	: KP1002545	Reported On	: **11-08-2025 16:36**
Sample ID	: 601613195	Referred By	: LBSH
Sample Type	: EDTA Whole Blood	Client Name	: MOOVE HEALTH CARE
		Processed At	: KPL A43

Test	Result	Unit	Reference Range
ABSOLUTE LYMPHOCYTE COUNT	: 2.02	x10^9/L	1.00 - 3.00
ABSOLUTE MONOCYTE COUNT	: **1.42**	x10^9/L	0.20 - 1.00
ABSOLUTE EOSINOPHIL COUNT	: 0.16	x10^9/L	0.02 - 0.50
ABSOLUTE BASOPHIL COUNT	: 0.10	x10^9/L	0.02 - 0.10
E.S.R. (Modified Westergren)	: **30.00**	mm at first hr	0 - 15.0

Method for DLC - Laser Fluorescence Flowcytometry / Light microscopy.
Pediatric reference ranges: For children, the Reference ranges are for age groups of 1year, 2-6 years & 6-12 years. The reference range for differential leucocyte count percentages (%)and RDW-CV% are for adults only. For specific ranges, consult a physician.

Reference Sedimentation rate (mm in 1 hour at 20 ± 3 deg C)

Men		**Women**	
17-50 yr	10 or <	17-50 yr	12 or <
51-60 yr	12 or <	51-60 yr	19 or <
61-70 yr	14 or <	61-70	20 or <
>70 yr	30 or <	>70 yr	35 or <

MC - 2712

Dr.N.Magoon
M.D. (Path)
Consulting Pathologist DMC
Reg.No-97859

"To Avail PHYSIOTHERAPY/ ECG/ NURSING SERVICES at your HOME, Call 7239-940000"

ON PANEL : **CGHS**, CAPF, DGEHS, ECHS, BHEL, INDIAN RAILWAY, NDMC, DELHI JAL BOARD, ONGC, NTPC, SAIL, BSF, SSB, CRPF, CISF, NSG, Assam Rifles

1. All reports are for interpretation by the treating doctor only and have to be viewed and correlated with clinical examination and other investivations. 2. All investivations have their limitation which is imposed by the limits to sensitivity and specificity of individual assay procedures as well as the quality of the specimen received by the laboratory. Isolated laboratory investigations never confirm the final diagnosis of the disease. 3. If the result (s) of tests are alarming or unexpected the doctor is advised to contact the lab immediately for needful and necessary action.

This Report is not subject to use for any medico-legal purpose

LABORATORY REPORT

Name	: **Mr.SATENDRA KUMAR**	Location	: KPL A43
Age/Gender	: **31 Y(s) /Male**	Registered On	: 11-08-2025 14:42
Reg No	: 1108253311	Collected On	: 11-08-2025 14:42
Lab ID No	: KP1002545	Reported On	: 11-08-2025 16:50
Sample ID	: 601613195	Referred By	: LBSH
Sample Type	: Serum	Client Name	: MOOVE HEALTH CARE
		Processed At	: KPL A43

Test	Result
HIV I+II	: NON REACTIVE
Method : Immunochromatography	

COMMENTS:-

a. HIV TRI DOT has been developed and designed using gp-41, C terminal of gp120 & gp36 representing the immunodominant regions of HIV-1 & HIV-2 envelope gene structure respectively.

b. The HIV TRI -DOT Test is a visual rapid, sensitive and accurate immunoassay for the differential detection of HIV-1& HIV -2 antibodies (IgG) in Human serum or plasma using HIV-1 & HIV-2 Antigens immobilized on an immunofiltration membrane. The test is a screening test for anti- HIV-1 & HIV-2.

c. This is only a screening test. All samples detected reactive must be confirmed by using HIV Western Blot. Therefore for a definitive diagnosis, the patient~s clinical history, symptomatology as well as serological data, should be considered.

**** End Of The Report ****

MC - 2712

Dr.Sharin Verma
MBBS, M.D, DNB (Microbiology)
Consultant Microbiologist DMC
Reg.No - DMC/R/1214

Print Date : 12-08-2025 16:18

"To Avail PHYSIOTHERAPY/ ECG/ NURSING SERVICES at your HOME, Call 7239-940000"

ON PANEL : **CGHS**, CAPF, DGEHS, ECHS, BHEL, INDIAN RAILWAY, NDMC, DELHI JAL BOARD, ONGC, NTPC, SAIL, BSF, SSB, CRPF, CISF, NSG, Assam Rifles

1. All reports are for interpretation by the treating doctor only and have to be viewed and correlated with clinical examination and other investivations. 2. All investivations have their limitation which is imposed by the limits to sensitivity and specificity of individual assay procedures as well as the quality of the specimen received by the laboratory. Isolated laboratory investigations never confirm the final diagnosis of the disease. 3. If the result (s) of tests are alarming or unexpected the doctor is advised to contact the lab immediately for needful and necessary action.

This Report is not subject to use for any medico-legal purpose